



Additional Copy to:

Clinical Diagnosis & History:

Biopsy-proven papillary thyroid carcinoma.

Specimens Submitted:

- 1: Right paratracheal lymph nodes (fs)
- 2: Left neck dissection level 3
- 3: Left neck dissection level 4
- 4: Delphian lymph node
- 5: Total thyroid
- 6: Central compartment neck nodes

DIAGNOSIS:

1. Right paratracheal lymph nodes (fs):

Part # 1

Lymph Node Dissection:

Metastatic papillary thyroid carcinoma
Number of lymph nodes examined: 1
Number of lymph nodes with metastatic disease: 1
Size of the largest lymph node involved by tumor: 0.8cm.
Size of the largest metastatic focus : 0.8 cm.
Extranodal extension is not identified

2. Left neck dissection level 3:

Part # 2

Lymph Node Dissection:

Metastatic papillary thyroid carcinoma
Number of lymph nodes examined: 1
Number of lymph nodes with metastatic disease: 1
Size of the largest lymph node involved by tumor: 1cm.
Size of the largest metastatic focus : 0.9 cm.
Extranodal extension is not identified

100-0-3

Path: carcinoma, papillary, thyroid 8260/3

CRCF: carcinoma, papillary, tall cell variant
8344/3

Site: thyroid, nos c73.9

10/27/12

3. Left neck dissection level 4:

Part # 3

Lymph Node Dissection:

Metastatic papillary thyroid carcinoma
Number of lymph nodes examined: 9
Number of lymph nodes with metastatic disease: 1
Size of the largest lymph node involved by tumor: 0.8cm.
Size of the largest metastatic focus : 0.5 cm.
Extranodal extension is not identified

Dx discrepancy form
States tall cell variant,
per TSS.

10/27/12

4. Delphian lymph node:

Part # 4

Lymph Node Dissection:

Metastatic papillary thyroid carcinoma

Number of lymph nodes examined: 2

Number of lymph nodes with metastatic disease: 2

Size of the largest lymph node involved by tumor: 0.3cm.

Size of the largest metastatic focus : 0.1 cm.

Extranodal extension is not identified

5. Total thyroid:

Part # 5

Tumor Type:

Multifocal papillary carcinoma, classical and microcarcinoma types

Histologic Grade:

Well differentiated

Mitotic Activity:

Not identified

Tumor Necrosis:

Not identified

Tumor Location:

Left lobe and isthmus

Tumor Size:

The tumor foci greatest size range from less than 0.1 to 1.4 cm

Tumor Encapsulation:

Partially and non-encapsulated foci

Blood Vessel Invasion:

Not identified

Extrathyroid Extension:

Not identified

Surgical Margins:

Tumor is present at inked and cauterized posterior margin

Non-Neoplastic Thyroid:

Exhibits nodular hyperplasia

Parathyroid Glands:

Identified

one unremarkable

Lymph Nodes:

Metastatic papillary carcinoma involves one of three lymph nodes (1/3). The lymph node involved by metastasis and the focus of metastasis measure 0.1 cm in greatest size. There is no extracapsular extension.

6. Central compartment neck nodes:

Part # 6

Lymph Node Dissection:

Metastatic papillary thyroid carcinoma

Number of lymph nodes examined: 9

Date Reported:

Addendum Diagnosis

2. Left level 3 lymph nodes; dissection:

- The remainder of the specimen was submitted and an additional six lymph nodes are identified and one of those shows metastatic papillary carcinoma (1/6). The lymph node measures 0.6 cm in greatest size and the metastatic focus measures 0.2 cm in greatest size. No extranodal extension identified in that lymph node.

Intraoperative Consultation:

1. Right paratracheal lymph nodes (fs): Metastatic papillary carcinoma
Permanent Section: same

Criteria	Yes	No
Diagnosis Discrepancy	Path OX DE AH. ✓	
Primary Tumor Site Discrepancy		✓
HPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Sync/Chronous Primary		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	RB	Date Reviewed: 10/16/12

Number of lymph nodes with metastatic disease: 7
Size of the largest lymph node involved by tumor: 0.9cm.
Size of the largest metastatic focus : 0.9 cm.
Extranodal extension is identified

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Gross Description:

[REDACTED]

1). The specimen is received fresh, labeled, "Right paratracheal lymph node", and consists of a lymph node measuring 1.0 cm in diameter. The lymph node is bisected and entirely frozen for intraoperative consultation.

Summary of sections:
EFS - entire lymph node

[REDACTED]

2) The specimen is received fresh, labeled "left neck dissection level 3" and consists of a portion of fibromembranous and fatty soft tissue measuring 4.5 x 3.0 x 0.8 cm in aggregate. A 0.9 x 0.6 x 0.4 cm gray-black nodule is identified. The lymph node is bisected and entirely submitted.

Summary of sections:
BLN - lymph node dissected

[REDACTED]

3) The specimen is received fresh, labeled "left neck dissection level 4" and consists of red-tan to yellow-tan aggregate of soft adipose tissue measuring 2.8 x 2.7 x 1.0 cm. Nine lymph nodes are identified, ranging from 0.3 to 1.0 cm. The lymph nodes are entirely submitted.

Summary of sections:
LN - lymph nodes (7)
BLN - bisected lymph nodes (2)

[REDACTED]

4) The specimen is received fresh labeled, "Delphian node", and consists of two pieces of tan soft tissue measuring 0.8 x 0.4 x 0.2 cm in aggregate. Entirely submitted.

Summary of sections:
U-- undesignated

5) The specimen is received fresh, labeled "total thyroid, which marks right upper pole" and consists of a thyroid weighing 20.0 g. The right lobe measures 4.9 x 2.8 x 1.4 cm, the left lobe measures 4.5 x 2.3 x 1.5 cm and the isthmus measures 2.3 x 0.8 x 0.7 cm. The external surface is covered by a thin fibrous capsule. The posterior surface of the specimen is inked black, and the anterior is inked blue. Sectioning reveals four white-tan firm rubbery encapsulated nodules in the left lower pole, ranging from 0.4 up to 1.4 cm in greatest diameter, and abutting the anterior and posterior inked margins. The remaining thyroid parenchyma is red tan. Representative sections of the specimen submitted for TPS. The remaining tissue is serially sectioned and entirely submitted.

Summary of sections:

L(1-8) - left lobe, superior to inferior, with nodules in L2-L8
RL - right lobe, superior to inferior
IS - isthmus

6) The specimen is received fresh, labeled "central compartment neck nodes" and consists of a 3.4 x 3.0 x 0.9 cm aggregate of yellow-tan hemorrhagic lobulated adipose tissue. Multiple lymph nodes are grossly identified, ranging from 0.3 up to 0.8 cm. The lymph nodes are entirely submitted.

Summary of sections:

LN - lymph nodes

BLN - bisected nodes

Summary of Sections:

Part 1: Right paratracheal lymph nodes (fs)

Block	Sect. Site	PCs
1	EFS	2

Part 2: Left neck dissection level 3

Block	Sect. Site	PCs
10	ADD	10
1	BLN	2

Part 3: Left neck dissection level 4

Block	Sect. Site	PCs
2	BLN	4
2	LN	7

Part 4: Delphian lymph node

Block	Sect. Site	PCs
1	U	2

Part 5: Total thyroid

Block	Sect. Site	PCs
2	IS	4
1	L1	2
1	L2	1
1	L3	1
1	L4	1
1	L5	1
1	L6	1
1	L7	1
1	L8	1
8	RL	12

Part 6: Central compartment neck nodes

Block	Sect. Site	PCs
4	BLN	4
2	LN	6

Procedures/Addenda:

Addendum

Date Ordered:
Date Complete:

Status: Signed Out

Pathologic Diagnosis Discrepancy Form

Instructions: The TCGA Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for TCGA is inconsistent with the diagnosis provided on the Case Quality Control Form completed for the submitted case.

Tissue Source Site (TSS): _____ TSS Identifier: _____ TSS Unique Patient Identifier: _____

Completed By (Interviewer Name on _____)

Completed Date: _____

Diagnosis Information

#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	<i>Classical papillary thyroid carcinoma</i>	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for TCGA, as reflected on the CQCF.	<i>Tall cell variant papillary carcinoma</i>	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form

3	Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form.	<i>Intra-observer variability in the assessment of the proportion of tall cells.</i>	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
4	Name of reviewing Pathologist or Biorepository Director		Provide the name of the pathologist who reviewed this case for TCGA.

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

Director

Date

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

Date